



Nursing Practice Guideline

Bladder Management and Urinary Catheterisation.

Reference #: NPG 44

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Nursing

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Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Bladder Management and Urinary Catheterisation . It ensures the delivery of a uniform, integrated multidisciplinary approach with management which is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒

Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Principles

Observe the five moments for hand hygiene. Always follow aseptic technique and use the risk assessment outlined in the SCGOPHCG Infection Prevention and Management Aseptic Technique Policy. This must include use of appropriate Personal Protective Equipment (PPE) to prevent contamination and mucosal or conjunctival splash injuries. Patient identification must be checked when matching patient's identity to care, therapy and services.

All personnel who insert and/or manage urinary catheters must adhere to the Work Health and Safety Act 2020.

1.0 Bladder Management

1.1 Medical Requirements

Some patients may need modification of accepted parameters due to known larger bladder capacity and/or known chronic urinary retention. These patients should be reviewed by the medical or continence service to reduce inappropriate catheterisation.

Patients presenting with total urinary retention (i.e. not passing any urine) require immediate clinical assessment and medical review.

1.2 Safety Information

Post operative patients are at high risk of developing Post Operative Urinary retention⁽¹⁾. Close monitoring of bladder with bladder scanning is recommended to reduce risk of bladder stretch injuries.

Every patient with a bladder stretch injury (>1000mls) should have a CIMS form completed and reviewed by the Continence Service and/or medical team.

1.3 Nursing Management

Patients experiencing Urinary Incontinence (UI).

- New or worsened existing UI – check post void bladder scan (PVBS) and urinalysis reporting any abnormalities to the treating team. If no infection or post void residual noted commence 3 days strict fluid chart (FBC) or 3-day Bladder Assessment chart and contact Continence service if required.
- Existing UI – discuss with patient if UI worsened, whether investigated previously, or if under pad subsidy scheme. Can have e-referral to SCGH Continence for review if indicated.
- **Monitor patients skin integrity** daily due to risk of developing Moisture Associated Skin Damage. Please refer to the SCGOPHCG Wound Management Nursing Practice Guidelines for best practice.

The normal voiding pattern is: 4-6 voids during the day and up to twice overnight. Normal bladder capacity is between 400-600mls, with sensation to void at approximately 200-300mls. Patients should void within a 4-6 hour period.⁽²⁾

Bladder scanning: Allows closer monitoring and assessment of patient's bladder function which leads to earlier interventions, reduce bladder stretch injuries and unnecessary instrumentation.^(1, 3-5)

- Obtaining a **baseline Post Void Bladder Scans (PVBS)** on admission and preoperatively identifies patients with **Chronic Urinary Retention (CUR)** a gradual, usually non painful ,incomplete emptying of the bladder⁽³⁻⁵⁾.
- Attending to regular bladder scanning and PVBS helps identifies those at risk of **Acute Urinary Retention (AUR)** a sudden inability to empty the bladder, usually painful and can be distressing. ⁽³⁻⁵⁾
- It is general accepted that a **PVBS > 300mls may be an indicator of urinary retention**. It should be investigated to identify cause, whether acute or chronic, its significance and treated accordingly.⁽³⁻⁵⁾

Signs and symptoms of acute urinary retention⁽³⁻⁵⁾:

- The sensation of a full bladder/bladder pain but patient unable to void or reporting difficulty in completely emptying the bladder compared to baseline.
- Small frequent voids with increasing PVR.
- Patient experiences urinary incontinence which can be sign of overflow incontinence (when the bladder is over full).

Medical history that increases risk of urinary retention ⁽³⁻⁵⁾

- Neurological conditions
 - Brain tumours, CVA, intracerebral haemorrhage, degenerative diseases such as Alzheimer's
 - Spinal cord injury or other conditions of the spinal cord
 - Injury of the nerves within the pelvis such as post-operative (hysterectomy, anterior resection) or radiation
 - Parkinson's disease, Multiple Sclerosis, Diabetic neuropathy.
- Previous bladder stretch injury such as chronic urinary retention from bladder outlet obstruction or a missed episode of urinary retention.
- Bladder Outlet Obstruction such as pelvic organ prolapse, gynaecological cancers, urethral stricture disease, prostate enlargement, and prostate cancer. Patients may report hesitancy, diminished urinary stream, straining to void, feeling of incomplete emptying and nocturia.

Reversible factors which may predispose to urinary retention:

- Recent surgery.
- Constipation.
- Urinary tract infection.
- Confusion and delirium.
- Pain.
- Immobility.
- Medications including anticholinergics, antihistamines, alpha adrenergic agonists, antipsychotics, sedatives and analgesics.
- Obstruction such as blood clots in the bladder.

Treatments:

- If practical remove reversible factors and reassess.
- Urinalysis
- Decompress bladder with intermittent catheter (IMC) or indwelling catheter (IDC).

Post insertion of an indwelling catheter, a trial of void (TOV) can be planned:

- If the team is confident that there has been sufficient address of factors such as treatment of constipation, delirium, sepsis, and any complicating medicines are withheld.
- Or on the advice of the Continence service or Urology. Note: OPH Surgical Service arranges all TOVs required following an inpatient elective admission in consultation with OPH Urology Consultant/ Registrar.

1.4 BLADDER MANAGEMENT FLOW CHART

TOV CHECKLIST

- **BOWELS OPENED?** If not resolve before commencing TOV.
- **Is it 06:00?** Early timing of TOV provides improved outcomes for patients.
- **If IDC was inserted for retention of urine** has the cause been identified and resolved or treated?
- **Ensure you have read page 2 and 3 of bladder management guidelines before commencing TOV**

RECOGNISE

THAT:

- 4 hourly monitoring of Bladder scans is recommended. Frequency should increase as volume on bladder scan increases to reduce risk of bladder stretch injury.
- Contraindications for bladder scanning are open skin, wounds, or staples in the supra pubic region. Ascites and pregnancy can affect the results as the scanner detects all collections of fluid.
- If there is doubt regarding accuracy of bladder scan an intermittent catheter should be performed.

Action is required if:

- Post void bladder scan is >500mls
- No void and bladder scan >650mls

IF PATIENT HAS SYMPTOMATIC, PAINFUL RETENTION DO NOT DELAY DECOMPRESSING BLADDER WITH A CATHETER, IRRESPECTIVE OF VOLUME ON BLADDER SCAN.

If no void in 4 hours

1. **Discuss with patient/carer their normal voiding pattern**
2. If appropriate to clinical condition encourage patient to try to void spontaneously by getting patient up to toilet, or as upright as possible on bed pan/bottle and provide **privacy if safe** to do so.
3. **If able to void**, perform Post Void bladder Scan (PVBS) within 15 **minutes** of void.
If elevated encourage double void and review INVESTIGATION section
4. **If unable to void** perform a random bladder scan (RBS)
If bladder volume elevated refer to ACT section
If bladder volume low - review fluid status and consider medical review if concerned

A trial of void is considered passed when for 2 consecutive voids:

1. The voided volume is greater than any residual.
2. Any residual volume is <200mls.

If this is not the case review INVESTIGATION section.

INVESTIGATE

What can be causing the urinary retention/high post void residual?

Is it Acute or Chronic?

- Review medical history and assess any reversible factors (see page 3)
- Known or suspected enlarged prostate?
- Review medication charts – any medications known to cause retention? Any missed medications i.e. alpha blockers for BPH.
- Review bowel chart – patient constipated? Remember frequent liquid stool could be an indication of overflow constipation.
- Check Urinalysis or midstream urine done during the admission.
- Review admission PVBS. Any previous history of urinary retention?

Discuss with Medical team, Continence team or Pharmacist to mitigate potential causes of urinary retention and move on to ACT section.

ACT

Suggested treatments:

- If practical remove reversible factors (see page 3) and reassess.
- Give any treatment prescribed by doctors i.e. aperients, pain relief etc.
- Decompress bladder with IMC or IDC and document as per page 10.
- Refer to Continence for patient assessment/send e referral.

An intermittent catheter is the reasonable action if:

1. A reversible factor can be treated within a 24-hour period
2. To comply with ward specific policies i.e. post-acute stroke
3. Planned for discharge and able to manage Clean intermittent Self Catheterisation

An indwelling catheter is the appropriate action for:

1. ALL Bladder stretch injuries (>1000mls). A CIMS form should be completed and referred to Continence
2. No spontaneous void and bladder scan >650mls (unless a plan is in place to manage this)
3. Patient has had IMC's and no spontaneous voids in 24-hour period.
4. Planned for discharge and unable to manage Clean Intermittent Self Catheterisation.

2.0 Urinary Catheterisation (Urethral)⁽⁶⁻¹²⁾

2.1 Medical Requirements

The parameters for catheterisation can be altered by medical staff or Continence service on an individualised patient basis.

2.2 Safety Requirements

- In post operative Urological patient – always consult the Urology Registrar prior to catheterisation.
- Patients within 6 weeks of RADICAL PROSTATECTOMY, should ONLY be catheterised by Urologist.

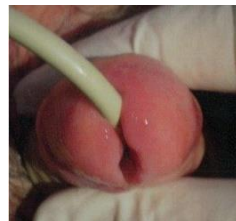
Catheterisation is NOT permitted by nurses in patients who have:

- Already had two unsuccessful attempts at catheterisation (one being a senior nurse).
- Required a catheter introducer or urethral dilator.
- A fractured pelvis or evidence of a ruptured urethra such as blood present at the urethral meatus and pelvic trauma.

The nurse must notify the Medical Officer immediately if:

- There is evidence of heavy bleeding during or post catheterisation.
- There is no evidence of urinary drainage within one hour of catheterisation.
- There is >200mls output for 2 consecutive hours post catheterisation as may be indicative of Post Obstructive Diuresis.

Failure to correctly secure a catheter with a securement device such as an adhesive securement sticker or catheter securement strap, can lead to pain, urethral erosion, pressure sores and increase the risk of infection and accidental dislodgment, which can cause severe urethral trauma.



Catheter Associated Urinary Tract Infections (CAUTI's) are one of the most common Healthcare Associated Infections (HAI's) causing increased length of stay and poorer outcomes for patients. Catheter use should therefore be rationalised, and ongoing requirement assessed daily to facilitate early and appropriate removal.

2.3 Nursing Management

Indwelling Catheter Types:

- 5-10mL balloon catheters are the catheter of choice for routine catheterisation.
- 30mL balloon catheters may be required for urological patients
- Routinely used catheters are lubricious/hydrogel- coated catheters with a latex core which can remain in situ for up to 6-12 weeks. i.e., Bard Biocath. Changed 6 weekly within hospital setting. **On imprest.**
- 100% silicone catheters can be used for IDC 's and SPC's and for those with Latex sensitivities. Varying indwelling times dependant on manufacturer.
- Silver coated catheters i.e. (Bardex™ I.C.) are occasionally used in patients with frequent CAUTI's and must be changed every 28 days. There is limited evidence that they reduce infection.

Silicone and infection control catheters can be accessed: SCGH: from the Continence service, mobile number 0482770662, or after-hours CNS. OPH: are available on special order via the ward Nurse Manager.

Blocked catheters can be caused by encrustations, debris/bladder stones or constipation.

- Ensure catheter tubing and drainage bag tubing are not kinked, and drainage bag is below bladder level.
- Discuss with medical team/continence service re-potential causes.
- **Changing the catheter is considered best practice** as frequent flushing of catheters is not recommended due to risk of CAUTI. Can also increase lumen size to allow better drainage.
- Patients with recurrent blockages should be assessed by the Continence service.

Bypassing catheters – can be caused by blocked catheter, CAUTI, bladder spasms, constipation, poorly secured catheter, and irritation from a large catheter.

- Ensure catheter tubing and drainage bag tubing are not kinked, drainage bag is below bladder level and catheter is well secured
- Assess patient for causes and discuss with medical team/continence service if required.
- Consider changing catheter down in size if appropriate.

Urine Specimen Collection - If urosepsis is suspected and a urine sample is required **discuss with medical staff how they would like sample taken.**

- A sample taken from a catheter which has been in situ for >48 hours is not recommended as it will be **colonised with bacteria** providing a potentially misleading result.
- Best practice recommends obtaining a fresh urine sample, from a **newly inserted catheter**, and document so on the pathology form, this may be contraindicated in some cases i.e. **Prostatitis – confirm with medical team.**
- If advised not to changed - collect specimen directly from the sampling port on the catheter bag tubing and not directly from the bag. Ensure to document of pathology form that it is a catheter sample of urine.

Catheter securement devices available.



Correctly securing the catheter with a securement device and alternating to other leg every second day, can reduce the risk of urethral erosion, pressure sores, and reduce CAUTI's.

Adhesive securement devices can be found on all ward areas. Ensure a supply is sent home with patient if discharged with a catheter.



Securement straps are advisable for patients with fragile skin or those likely to have a catheter for longer length of time.

They can be found on imprest via CNS/ward manager (order no 199988A) or through Continence service.

Catheter Valves



The use of a catheter valve allows for normal bladder filling and emptying of the bladder. This reduces the risk of bladder wall trauma and can reduce blockages, as the bladder fills naturally, does not collapse around the catheter, and allows for normal flushing of the bladder. It can also allow more discretion and comfort for the patient.

The patient must empty at sensation to void or at 3-4 hourly intervals throughout the day, ensuring the bladder does not fill more than 500mls. At night, the use of a large capacity overnight bag is used if there is concern about overdistention of the bladder and should be connected to the end of the catheter valve.

Catheter valves can be considered for patients with recurrent blocked catheters. Contact Continence Service for patient assessment. (order number 199985W)

Contact Continence service for individualised patient assessment as their use is contraindicated in patients with poor dexterity, cognitive impairment, existing ureteral reflux or renal impairment, and poor bladder compliance due to risk of renal injury and sepsis.

2.4 Urinary Catheterisation Male and Female (Urethral)⁽⁶⁻¹³⁾

- Use the smallest bore catheter consistent with good drainage, to minimise bladder neck and urethral trauma, unless otherwise clinically indicated.
- The use of 12 Fr catheters should be avoided due to poor drainage, and risk of blockages.

Clear urine, no sediment or haematuria	14 Fr	Male and Female
Slight cloudy urine, light haematuria with or without small clots, no or mild sediment	16Fr	Male and Female
Moderate to heavy sediment, haematuria with moderate clots	18 Fr	On advice of medical staff
Heavy haematuria requiring flushing	20-24 Fr	On advice of medical staff

Requirements

- Catheter pack
- Wet Strength Bag.
- Personal Protective Equipment.
- Sterile gloves.
- Absorbent sheet.
- Catheter of suitable size and type noting balloon size.
(intermittent catheterisations use Nelaton catheter)
- 1 x sterile local anaesthetic gel syringe (a second may be required in difficult catheterisations)
- Catheter preparation solution or 30mls sachet 0.9% sodium chloride.
- 10 mL ampoules of sterile water appropriate for balloon size.
- Sterile syringe appropriate for the catheter balloon size.
- Sterile, closed system Urinary drainage bag and holder
- Sterile urine specimen collection tube/yellow screw top container (optional if sample required)
- Clean plastic jug or urinal
- Catheter fixation device

Catheterisation – Female and male

Procedure

1. Explain procedure to the patient and obtain verbal consent.
2. Check for any sensitivities to latex, lignocaine, or chlorhexidine.

3. Perform hand hygiene.
4. Clean the trolley using a combined detergent/disinfectant wipe. Allow to dry.
5. Gather equipment.
6. Position trolley at side of bed which allows insertion of catheter with dominant hand
7. Perform hand hygiene.
8. Assist patient into supine position with knees bent outwards (Female) legs extended (male)
9. Perform Hand Hygiene.
10. Place the absorbent sheet under patient.
11. If required wash the peri urethral meatus with soap and warm water.
12. If absorbent sheet is wet, replace with clean sheet under the patient. Position the patient to enable a clear view of the urethral meatus.
13. Perform hand hygiene.
14. Open catheter pack using corners.
15. Open remaining equipment onto aseptic field using a non-touch technique.
16. Perform hand hygiene and dry with sterile towel.
17. Don sterile gloves and organise equipment on sterile field.
18. Using a non-touch technique place catheter in sterile kidney dish.
19. Apply small amount of lignocaine gel to catheter tip.
20. Pour sterile water into the second gallipot.
21. Drape patient with sterile lithotomy sheet over patient, exposing genitals:

Female	Male
22 - Clean left and right labia majora, with chosen cleaning solution, 1 swab for left and right.	<u>22-Retract foreskin</u> if patient is uncircumcised. Clean urethral orifice with cleaning solution chosen and swabs provided (glans, penis, and prepuce).
23 - With non-dominant hand separate labia minora, with dominant hand clean left and right labia minora and urethral orifice, 1 swab for each.	23 -Use the non-dominant hand to hold the penis firmly after wrapping in second sterile towel and in an upright position (perpendicular to abdomen).
24 - Instil lubricating gel into urethral meatus	24-Instil lubricating gel local anaesthetic into urethral meatus. To prevent the lubricant seeping out gently pinch the end of the penis.

25. Transfer kidney dish containing catheter to drape between patient's thighs.
26. Using forceps, insert catheter into the urethra using a non-touch technique until urine flows.
27. In Females - if unsuccessful with first attempt (the catheter has slipped into the vagina), leave catheter in-situ as a guide and try again with second sterile catheter.
28. **Completely decompress the bladder** and document amount drained in mls on catheter insertion sticker and fluid balance chart
Intermittent catheter - Ensure the bladder is empty before removing the catheter by removing slowly until no urine flows; supra-pubic pressure may be required, prior to the removal of the catheter.
Replace retracted foreskin if applicable to prevent paraphimosis.

Indwelling Catheter - Always insert the catheter till the bifurcation ("Y" connection) is at the urethral meatus and free flowing urine is noted before inflating the balloon. There should be no resistance when inflating the balloon.

Replace retracted foreskin if uncircumcised to prevent paraphimosis.

29. Connect the catheter to a **sterile** closed system drainage bag using a non-touch technique.
30. **Apply catheter fixation device** to upper thigh ensuring there is sufficient tubing length to allow for natural movement allowing the patient to be able to sit down or stand up without the catheter pulling, and to allow for nocturnal tumescence (erections).
31. Dispose of waste and remove PPE.
32. Perform hand hygiene.
33. Clean the trolley using a combined detergent/disinfectant wipe
34. Perform hand hygiene
35. **Complete Indwelling Urinary Catheter Details sticker** documenting in the Inpatient notes:
 - Rationale for insertion of catheter including bladder scan volume if the patient will be undergoing a Trial of Void (TOV).
 - Volume of sterile water used to inflate the balloon.
 - Colour, odour of urine and if a CSU was sent.
 - Catheter type, size, date of insertion, batch number, expiry date.
 - **Amount drained at insertion:**
 - This is essential for the patient admitted with urinary retention as it forms a guide of when to perform a trial of void.
 - If volume is >1000mls ensure hourly urine measures and be alert for post obstructive diuresis (protocol for this is located in the urology departmental guidelines).
 - Post obstructive diuresis is a medical emergency and is identified if there is >200mls for 2 consecutive hours and requires urgent medical review.⁽¹⁴⁾

2.5 Daily management of Indwelling Catheter – Female and Male ^(8, 9, 11-13)

- If permissible, individuals should be encouraged to drink sufficient fluid as this ensures a constant drainage and flushing effect and helps prevent blockage and encrustation.
- **Routine hygiene and catheter care** (during daily wash or shower) is needed to maintain meatal hygiene. In males this involves BD penile cleansing by pulling back the foreskin and washing the penis and catheter with warm soapy water, rinsing, and replacing foreskin in its usual position.
- Each shift ensure the **catheter securement device** is in correct position and tubing not taut to prevent Urethral erosion and/or pressure injury as well as allowing for nocturnal tumescence
- **Every 2nd day move catheter securement device** and catheter tubing to alternate leg to prevent pressure injury occurring.
- The urine drainage bag should be placed below the level of the bladder, but never be placed on the floor due to infection risks and trip hazard. Drainage bag holders are available.
- **If patient is mobile connect a leg bag.** Ensure straps are placed underneath bag with buttons to the front, so urine flow not obstructed. Straps can be cut to size to ensure secure fit. To maintain urine flow and prevent urine reflux, ensure leg bag is emptied when it is half full, this also prevents the weight of the bag causing potential urethral trauma.
- Ensure that the connection between the catheter and the drainage system is not broken, unless changing the leg bag in line with the manufacturer's recommendations (7 days)
- **Sterile 2 litre Night drainage bags** are connected to the base of the leg bag.
- Daily review of the need for IDC and plan for removal (early removal prevents

nosocomial Catheter Associated UTI (CAUTI)

3.0 Removal of Indwelling catheter^(8, 9, 15)

3.1 Medical Requirements

Removal of catheter should be done in consultation with medical staff as soon as medically possible

Anticholinergic/antispasmodic medications e.g., Oxybutynin, should ideally be discontinued 24 hours prior to catheter removal. It should not delay catheter removal for trial of void unless there are significant risks that catheter re-insertion will require urological intervention.

3.2 Safety Requirements

If the balloon fails to deflate, do not cut the catheter inflation port;

- **SCGH:** contact the Urology Registrar, or Continence Service, and at
- **OPH** contact the Medical Officer, and Continence Wound and Ostomy Clinical Nurse Consultant.

3.3 Nursing Management

Requirements

- Personal Protective Equipment
- Syringe – note size of balloon or consult sticker in the notes and choose appropriate size
- Wet Strength Bag

Procedure

1. Perform hand hygiene, don PPE.
2. Connect syringe gently to the catheter inflation port using a non-touch technique and allow water to **dispel passively** from the balloon to fill the syringe.
3. Remove catheter, discard waste into wet-strength bag and dispose in general waste.
4. Remove PPE and perform hand hygiene.
5. Document on fluid balance chart intake and voided volume.
6. Perform at least two post void bladder scans,
7. If the patient is being discharged, patient should be educated regarding signs and symptoms of Urinary Tract Infection and advised to follow up with GP if symptoms arise.
8. Patient should be educated on sign and symptoms of urinary retention (see page 3) and be advised to present to the nearest emergency department if this happens.

4.0 Supra-pubic Catheterisation^(8, 9, 16)

4.1 Medical Requirements

SCGH: Initial insertion is performed by a Medical Officer in theatre, the emergency department, the ward, or Endoscopy unit (Ward G75).

OPH: Insertion usually carried out in theatre.

4.2 Safety Information

First changes of Supra pubic catheters should only be done 6 weeks after insertion. Performed on G75 in SCGH by the Continence service or Urology and by Urology at OPH.

Post new catheter insertion or change report any excessive bleeding or decreased output to medical staff.

Supra pubic catheter changes should only be done by a competent nurse in the procedure, or under direct supervision of a nurse competent in the procedure. 2 nurses ideally should be available at time of procedure.

4.3 Nursing Management

- SPC changes are usually done 6 weekly but can be done earlier for management of infection and encrustations/ blockages, or longer if no complications.
- Dressings to SPC sites should only be used until track forms and no ooze from site to reduce risk of infection.
- Daily cleaning of SPC site should occur with soap and water.

Pre-Supra Pubic Catheter Change

- The bladder needs filling prior to SPC change as the bladder is collapsed around the catheter (unless using a catheter valve) This allows for easier removal and quicker identification of correct position. This can be done in two ways:
 1. Spiggot catheter for **30 minutes** prior to change. See pre procedure instructions.
 2. **If catheter valve** attached do not empty for **30 minutes** prior to change.

Note : Over distension should be **avoided** in patients with **Spinal Cord Injuries T6** and above as it can cause autonomic dysreflexia - a serious medical emergency.

Procedure

1. Perform hand hygiene, apply PPE
2. Rotate the catheter in the cystostomy site (to unstick the catheter from the tract) prior to removal.
3. Clean catheter drainage port with alcohol swab and allow to dry.
4. Remove bag and insert spigot. **Note time to ensure it is not spigotted for more than 30 minutes prior to procedure**
5. Discard waste.
6. Remove PPE and perform hand hygiene.
7. Administer analgesia if indicated.

Catheter change

Requirements

- Catheter pack
- Catheter preparation solution or 30mls sachet of 0.9% sodium chloride.
- 2 x syringes appropriate to catheter balloon size
- 10 mL ampoules sterile water appropriate to balloon size.
- Personal Protective Equipment including sterile gloves
- Wet strength bag
- Lubricant/anaesthetic gel -1 syringe
- Sterile closed system urine drainage bag
- Lubricious coated Biocath™ or 100% silicone catheter– same size as current catheter and 1 size smaller.
- Catheter securement device

Procedure

1. Inform patient of procedure and gain consent.
2. Check of any **latex, chlorhexidine, or lignocaine sensitivities**.
3. Perform hand hygiene.
4. Clean the trolley using a combined detergent/disinfectant wipe. Allow to dry.
5. Gather equipment.
6. Perform hand hygiene.

7. Open catheter pack using the corners.
8. Place remaining supplies onto sterile field. Only 1 syringe on field.
9. Perform hand hygiene.
10. Use syringe to passively deflate balloon and **leave in situ**.
11. Perform hand hygiene and dry hands with 1x sterile field. Don sterile gloves.
12. Organise sterile field for procedure
13. Using a non-touch technique place catheter in kidney dish, lubricate tip with lignocaine gel.
14. Draw enough water for the balloon into the syringe.
15. Clean around SPC site with catheter preparation solution and sterile gauze/balls using a non-touch technique.
16. Insert lubricant/anaesthetic gel using a non-touch technique.
17. 1 nurse procedure – with non-dominant hand use second sterile towel around old catheter and remove the old catheter. Insert the new catheter with dominant hand, ensuring no cross contamination.
18. 2 nurse procedure – second nonsterile nurse applies nonsterile gloves and removes the old catheter from under the clean field, the second sterile towel can also be used to aid removal.
New catheter inserted by sterile nurse, ensuring no cross contamination.
19. If having difficulty removing the catheter brace the skin at the supra-pubic site firmly and pull the catheter out (after warning the patient). This occurs commonly with suprapubic catheters as the balloon does not deflate flat and can form a pucker.
20. Gently insert new catheter into suprapubic opening approximately 15cms using a non-touch technique. Ensure free flowing urine prior to inflation of balloon.
21. Inflate balloon with 10mLs (or appropriate volume) of sterile water using a non-touch technique.
22. Connect sterile closed system leg drainage bag using a non-touch technique.
23. Apply catheter securement device.
24. Dispose of waste and remove PPE.
25. Perform hand hygiene.

If any issues arise with removal or insertion of SPC contact Continence service in office hours or Urology out with these times.

4.4 Supra-Pubic Catheter Removal

Procedure

1. Perform hand hygiene and apply PPE.
2. Deflate balloon in catheter and remove gently using a non-touch technique.
3. Seal the supra-pubic catheter site with gauze, combine and occlusive dressing using a non-touch technique.
4. Remove PPE, perform hand hygiene.
5. Leave intact for 24 hours if possible. If excessive leakage occurs change and reinforce dressing.
6. Ensure puncture site remains clean and dry.
7. If patient has not voided within 4 hours of removal, ensure bladder scanning is performed and monitor for signs of urinary retention.

5.0 Discharge of patient with Indwelling or Supra-pubic catheter

5.1 Medical Requirements

Antispasmodic/anticholinergic medications should ideally be discontinued 24 hours prior to catheter removal. It should not delay catheter removal for trial of void unless there are significant risks that catheter re-insertion will require urological intervention.

5.2 Nursing Management

Discharge of patient with **long term IDC/SPC** changed in hospital.

- Ensure usual nursing service that attends to catheter changes is informed of change date on discharge. **Ward staff responsibility.**

Discharge of patient with **new long-term IDC** (If difficult insertion or <65years old see below)

- Patient will require 6/52 IDC changes in the **community** with either:
 1. Silverchain (if >65 years old and not receiving level 3 or 4 package)
 2. NDIS provider – if Continence is part of their package, if not then Silverchain until it is added.
 3. Usual Home care nursing provider if level 3 or 4.
 4. Nursing home - Residential Care line can provide support if required to staff

It is the wards responsibility to organise on discharge. Ensure provider is informed of date of insertion, size of catheter and any other relevant information i.e. risk factors.

If difficult insertion or patient **<65 years old and does not qualify for any of the above categories**, Continence at SCGH will attend to 6/52 changes. Send e- referral to SCGH Continence with all relevant information, including date of insertion and catheter size, and any risk factors.

Discharge of patient with **new SPC** insertion.

- Ensure that any **sutures** (if present) **are removed one week** after insertion either by patient's own GP, or nursing service already in place.
- A dressing may be required for the first six weeks (only required if any ooze) – assess if this can be managed by patient/carers or requires home nursing service.
- Patient will require **first change in hospital** by the Continence service 6/52 from insertion. During hours – contact Continence service or send e referral for inpatient review. Out of hours - please send e referral to SCGH Continence service providing insertion date, catheter size and any relevant information and risk factors.

Discharge of patient with temporary IDC for Trial of Void (TOV)

- **Continence Service should be informed of all patients for TOV BEFORE day of discharge**, but it should not delay a discharge. Contact Continence Service or send e referral for inpatient review.

TOV can be done with:

1. Homelink – Continence service must review before they will accept unless out of hours. **It is wards responsibility to complete referral paperwork.**
2. Silverchain - if out of catchment area for Homelink OR if the nursing staff identifies the patient's need for ongoing support and education with their catheter care. **It is wards responsibility to complete referral paperwork.**
3. Residential Care Line – if Nursing Home patient.
4. Short stay ward – if an hospital based TOV is requested by medical team or Continence Service.

Out of hours – please submit an e-referral to SCGH Continence Service and follow e-referral template to provide all relevant information.

5.3 Procedural Information (8-13, 15, 17)

Preparation for discharge for patients with IDC/SPC:

1. Attach a **leg bag** to the catheter using straps provided (**cut to size, strap underneath bag, buttons outwards**) to secure to the leg.
2. Secure catheter to upper thigh with **securement sticker** if short term or catheter **securement strap** (available from Continence or from Nurse Manager) if longer term. Ensuring catheter not pulled taut to allow for natural movement and free

- drainage or urine.
3. Supra- pubic catheter may be secured to the abdomen if patient prefers.
 4. Commence **education** of patient to ensure patient or carer able to manage catheter as **early as practical**.
 - Ensure patient knows to wash their hands (hand hygiene) before and after emptying, connecting, or changing their drainage bags.
 - Ensure able to empty leg bag when $\frac{1}{2}$ to $\frac{3}{4}$ full so weight not pulling down on catheter.
 - Ensure able to change leg bag when required i.e. weekly as per manufacturer instructions or if bag malfunction, and that it should remain a closed system at all other times to reduce the risk of infection.
 - Ensure able to attach and detach night bag to bottom of leg bag, knows to empty, rinse, and leave to dry for next night, and to position lower than bladder to allow free drainage of urine i.e. on floor in clean bucket.
 - Ensure able to move leg bag and securement sticker /strap to alternate leg second daily to prevent risk of pressure sore or erosion.
 - Ensure patient knows to have adequate fluid intake and avoid constipation which can cause urinary bypassing.
 5. **Supply to patient:**
 - Care of Catheter at Home information leaflet (found on ward or on SCGOPHCG HealthPoint)
[Catheter---At-home-with-your-catheter---A-guide-for-patients-and-carers.pdf \(health.wa.gov.au\)](https://www.health.wa.gov.au/healthpoint/At-home-with-your-catheter---A-guide-for-patients-and-carers.pdf)
 - Spare leg bag x2 (can use spare straps post showering to replace wet straps) – should be changed weekly.
 - Night bags x2
 - Spare catheter securement stickers if strap not provided.

6.0 Compliance, monitoring and evaluation

As this document replaces a long-standing guideline, SCGH Continence will deliver education and support to staff throughout the transition. SCGH will audit staff to assess if the changes have increased knowledge and changes in practice.

7.0 Related Documents

Legislation:

- Health Practitioner Regulation National Law (WA) Act 2010
- Medicines and Poisons Act 2014 and the Medicines and Poisons Regulations 2016
- Nursing and Midwifery Board of Australia (NMBA) Code of Professional Conduct for Nurses March 2018
- Work Health and Safety Act 2020

Policy:

- NMHS Consent to Treatment Policy
- NMHS Chaperone Policy
- NMHS Clinical Documentation Policy
- NMHS Patient Identification and Procedure Matching Policy
- NMHS Incident Hazard Reporting and Investigation Policy
- NMHS Healthcare Record Management Policy
- NMHS Infection Control Policy
- NMHS Personal Protective Equipment Policy
- SCGOPHCG Consent to Treatment Policy
- SCGOPHCG Infection Prevention and Management -Aseptic Technique Policy
- SCGOPHCG Infection Prevention and Management Manual- Specimen Collection Guideline

- SCGOPHCG Infection Prevention and Management Manual - Cleaning of Patient Care Equipment Policy
- SCGOPHCG Infection Prevention and Management Manual – Hand Hygiene Policy
- SCGOPHCG Clinical Handover - Communicating for Safety
- SCGOPHCG Patient Identification Policy
- SCGOPHCG Procedure Matching Policy

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9.0 Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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